

Seating Evaluation for Power Mobility Device (PMD)

1 Patient Data

1a) Patient Name :

1b) Addendum to Progress Notes (Date of Visit):

1c) Purpose of F2F visit ☐ Power Mobility Evaluation
☐ Follow Up

1d) Height (in.) _____ Weight (lbs.) _____

1e) Diagnoses *Diagnoses contributing to mobility limitations which significantly impair participation in mobility-related activities of daily living (MRADLs) such as toileting, feeding, dressing, grooming, & bathing in the home*

2 Compromised MRADLs

Mobility limitations impair the patient's independent ability of:	Cannot accomplish entirely	Cannot attempt without a heightened risk of morbidity/mortality	Cannot complete within a reasonable timeframe
going to bathroom to toilet	☐	☐	☐
going to kitchen to eat	☐	☐	☐
dressing	☐	☐	☐
grooming	☐	☐	☐
going to bathroom to bathe	☐	☐	☐

3 Functional Limitations

3a) Upper/Lower
Extremity Strength
(UE/LE)

Left Upper	/5	/5	Right Upper
Left Lower	/5	/5	Right Lower

0 (Zero): No muscle strength

1 (Trace): Too weak to move muscle

2 (Poor): Enough strength to move joints

3 (Fair): Complete ROM w/ no resistance

4 (Good): Complete ROM w/ moderate resistance

5 (Normal): ROM w/ strong resistance

3b) Range of Motion
(ROM)☐ Limited ROM☐ Limited Range of Extension☐ Limited Range of Flexion☐ WFL3c) Endurance | ☐ Poor | ☐ Strong | *Lack of performing repeated activities in a reasonable time limits mobility*3d) Fall Risk | ☐ Yes; Number of falls or "near falls-imbalance" in past 6 months: _____ | ☐ No No issue

3e) Pain

Location
Scale

UE

☐ Neck ☐ Shoulder ☐ Elbow ☐ Arm ☐ Hand
☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8 ☐ 9 ☐ 10Location
Scale

LE

☐ Back ☐ Hip ☐ Knee ☐ Leg ☐ Joints ☐ Foot
☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8 ☐ 9 ☐ 10

Pain in combination with compromised extremity strength, limited ROM, poor endurance, imbalance, and/or lack of coordination may result in the inability to weight-shift independently multiple times a day, for a sufficient duration each time to allow for reperfusion of the skin and muscles and thus may require pressure management

4 Lower cost alternatives considered to perform MRADLs during a typical day in the home

4a) Limitations resolved sufficiently & safely
using appropriately fitted **Cane** or **Walker**?☐ Yes☐ No; **Rules out Cane or Walker**4b) Sufficient UE function & ROM to self-propel
an optimally-configured* **Manual Wheelchair**?☐ Yes☐ No; **Rules out Manual Wheelchair**

**with the appropriate wheelbase, device weight, seating options,
and other appropriate non-powered parts*

4c) Is the patient able to safely transfer to and
from POV, operate the tiller steering system,
and maintain postural stability, while operating
Power Operated Vehicle (POV) in home?☐ Yes☐ No; **Rules out Scooter/POV**

5 Cognitive and Physical Impairment Status

5a) ☐ Intact ☐ Severely Impaired; ☐ Memory skills ☐ Concentration ☐ Vision ☐ Judgement ☐ Hearing ☐ Deformity5b) If **severely impaired**, does the patient have a caregiver available
who is unable to propel an optimally configured manual wheelchair,
but is willing and able to safely operate a power chair?☐ Yes; *Attendant control will help*☐ No

Seating Evaluation for Power Mobility Device (PMD)

6 Feature Selection - Single Power

6a) Inability to **weight shift** functionally and independently while seated due to pain, compromised strength, limited ROM, poor endurance, imbalance, coordination and/or obesity? ☐ Yes ☐ No

6b) Factors causing a high risk of developing pressure ulcers	☐ Limited Sensory Perception	☐ Friction & Shear	☐ Poor Nutrition
	☐ Very Limited Mobility	☐ Bed/Chair Bound	☐ Moisture
	☐ Low Blood Pressure	☐ Advanced Age	☐ Obesity
	☐ Hemodynamic Instability	☐ Neuropathy	☐ Diabetes

6c) Overall Risk | ☐ Severe* ☐ High* ☐ Moderate ☐ Mild

**Tilt will facilitate functional weight shift and reduce overall risk of developing pressure ulcers*

6d) Does the patient use **Intermittent Catheters**? ☐ Yes ☐ No

*If yes, is it difficult for the patient to **transfer** from chair to bed in a reasonable time?* ☐ Yes** ☐ No

6e) Muscle Tone | ☐ Spasticity** ☐ Joint Contracture ☐ Rigidity ☐ Hypotonia ☐ Normal

***Recline will facilitate to open seat to back angle for catheterization and/or to manage spasticity*

7 Feature Selection - Multi Power The combination of both **tilt and recline** is necessary for ...

☐ Better transfer	☐ Improving lower limb edema	☐ Managing orthostatic hypotension
☐ Achieving optimal pressure relief by combining 25°- 45° of tilt with 110°-115° of recline	☐ Optimizing position for respiration, eating, swallowing, and vision	☐ Allowing a dynamic seating position to permit the widest range of postural changes throughout the day for optimal MRADLs

8 Seat/Back Cushion Selection - Skin Protection and/or Positioning

8a) History of/Current **Pressure Ulcers**? ☐ Yes* ☐ No

If yes, Location of Ulcers: ☐ Lower Back ☐ Sacrum ☐ Buttock ☐ Hip

**Skin protection cushion will allow for weight redistribution and pressure relief*

8b) Absent/Impaired Sensation at the area of contact? ☐ Yes* ☐ No

8c) Presence of lower limb **Amputation**? | ☐ Yes; *significant postural asymmetry of lower limb* ☐ No

If yes; ☐ LT-Hip ☐ RT-Hip ☐ LT-AKA ☐ RT-AKA ☐ LT-BKA* ☐ RT-BKA*

**Stump support will help*

Positioning seat cushion will help to address postural asymmetries and/or orthopedic deformities

8d) Presence of **Significant Postural Asymmetries**? ☐ Yes* ☐ No

If yes; **Pelvis** ☐ Posterior Tilt ☐ Anterior Tilt ☐ Obliquity ☐ Rotation
Trunk ☐ Kyphosis ☐ Lordosis ☐ Scoliosis

**Positioning back cushion and accessories will help to address postural asymmetries and mid-line support*

9 Utilization and Benefits

9a) Need to Manage | ☐ Increased Tone ☐ Pressure reduction ☐ Edema
☐ Posture instability ☐ Mid-line trunk support

Management of above will be significantly improved by a power seating system w/tilt, recline and the appropriate accessories

9b) MRADLs will be significantly improved by use of PMD? ☐ Yes ☐ No

9c) Willing and able to use PMD safely? ☐ Yes ☐ No

9d) Hours of Use/Day ☐ > 2; *Adjustable height arm rests will facilitate transfer* ☐ < 2

9e) Is the intended **primary use** of the PMD within the home? ☐ Yes ☐ No

10 PMD Selection

PMD Base | ☐ Power Wheelchair ☐ Scooter (POV)

Features | ☐ Tilt** ☐ Recline** ☐ Tilt & Recline***

Cushions | ☐ Skin Protection Seat ☐ Positioning Seat ☐ Positioning Back

**Include electro-connect. btw controller and motor, headrest
**Include expandable controller, electro-connect. btw multi-power functions, wiring harness, headrest
* Include power elevating leg platform*

Accessories and drive control items will be identified and submitted by the supplier in the detailed SWO.

Comments:

I have no financial relationship to the supplier. I am a Licensed Certified Medical Professional trained to evaluate rehabilitation PMDs.

Practitioner Name |

Signature |

Date |

(print name)

(hand sign)

(hand date)